

SULIT



LAPORAN PERUBATAN MEDICAL REPORT

BUTIRAN PESAKIT PATIENT PARTICULARS

Nama pesakit <i>Name of patient</i>		:	Ahmad Bin Hassan	
No K/P I/C No.	Baru <i>New</i>	:	920315-10-5432	
	Lama <i>Old</i>	:	N/A	
No. Passport <i>Passport No.</i>		:	N/A	
MRN		:	MRN-2024-001001	
No. Pendaftaran <i>Reg. No.</i>	Penuh <i>Full</i>	:	REG-FULL-2024-0055	
	Sementara <i>Provisional</i>	:		
	Tiada <i>Null</i>	:		
Umur <i>Age</i>		:	32	
Jantina <i>Sex</i>		:	Lelaki <i>Male</i>	☒
			Perempuan <i>Female</i>	☐

Tarikh masuk wad atau menerima rawatan buat kali pertama <i>Date of admission or receiving treatment for the first time</i>	2024-05-10
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Tempat menerima rawatan *Place where patient received treatment*

☒	Jabatan Kecemasan <i>Emergency Department</i>
☐	Klinik Pakar <i>Specialist Clinic</i>
☐	Wad <i>Ward</i>

Tarikh discaj dari wad atau meninggal dunia <i>Date of discharge or death</i>	2024-05-10 (Same day discharge from ED)
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Disiplin <i>Discipline</i>	:	Perubatan Kecemasan / Emergency Medicine
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LAPORAN PERUBATAN MEDICAL REPORT

Sejarah <i>History</i>	:	• Presenting Complaints: Patient presents with productive cough, fever, and mild shortness of breath for 3 days. • History of Presenting Complaints: Symptoms started 3 days ago with a dry cough, progressing to a productive cough with yellowish sputum. Associated with subjective fever and chills. Shortness of breath is mild, occurring on exertion. No chest pain, hemoptysis, or wheezing. • Past Medical History: No known chronic illnesses (e.g., hypertension, diabetes, asthma). No previous hospitalizations or surgeries. No known drug allergies. • Family History: Father has hypertension. Mother is healthy. No family history of respiratory diseases or tuberculosis. • Social History: Non-smoker, occasional social alcohol use. Works as an office administrator. Lives with spouse and two children. • Occupational History: Office environment, no known exposure to respiratory irritants or sick contacts at work. • Review of systems: General: fatigue, malaise. Respiratory: cough, dyspnea. Cardiovascular: no palpitations, no edema. Gastrointestinal: good appetite, no nausea/vomiting/diarrhea. Genitourinary: normal. Central Nervous System: no headache, dizziness, or seizures. Musculoskeletal: no joint pain or swelling. • Medical Records reviewed: None available at the time of initial assessment.
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Including Presenting Complaints, History of Presenting complaints, Past Medical History, Family History, Social History and Occupational History, Review of systems, Medical Records reviewed

Pemeriksaan Fizikal <i>Physical Examination</i>	:	• General Assessment: Conscious, alert, cooperative. Appears acutely ill but not in severe respiratory distress. • Vital Signs: Temperature 38.2°C, Blood Pressure 125/85 mmHg, Heart Rate 98 bpm, Respiratory Rate 24 breaths/min, SpO2 94% on room air. • Eye: Conjunctivae not pale, sclerae anicteric. Pupils equal and reactive to light. • ENT: Oropharynx mildly congested, no tonsillar exudates. No cervical lymphadenopathy. • Oral Cavity: Moist mucous membranes. • Respiratory System: Chest expansion symmetrical. Percussion resonant. Auscultation reveals bilateral coarse crepitations in the lower lung fields, more prominent on the right. Scattered rhonchi heard. No wheezes. • Cardiovascular System: S1S2 heard, no murmurs, rubs, or gallops. Capillary refill time < 2 seconds. Peripheral pulses palpable and symmetrical. • Abdomen: Soft, non-tender, non-distended. Bowel sounds present. No organomegaly. • Genitourinary: Not examined. • Central Nervous System: GCS 15/15. Cranial nerves intact. No focal neurological deficits. • Musculoskeletal: No clubbing, cyanosis, or edema. No joint swelling or tenderness. • Mental Health Status: Anxious about current symptoms, otherwise oriented to time, place, and person.
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Including general assessment, Eye, ENT, Oral Cavity, Respiratory System, Cardiovascular System, Abdomen, Genitourinary, Central Nervous System, Musculoskeletal, Mental Health Status and Others

Keputusan ujian makmal dan radiology <i>Summary of investigations</i>	:	• Full Blood Count (FBC): White Blood Cell (WBC) count $16.2 \times 10^9/L$ (elevated), Neutrophils 85%, Lymphocytes 10%. Hemoglobin (Hb) 14.5 g/dL. Platelets $280 \times 10^9/L$. • C-Reactive Protein (CRP): 110 mg/L (markedly elevated). • Renal Profile & Electrolytes: Within normal limits. • Liver Function Test: Within normal limits. • Chest X-ray (CXR): Consolidation noted in the right lower lobe and patchy infiltrates in the left lower lobe, consistent with bilateral pneumonia. No pleural effusion. • Sputum Gram Stain: Gram-positive cocci in clusters. Culture pending.
Diagnosis <i>Diagnosis</i>	:	• Community-Acquired Pneumonia (CAP), likely bacterial etiology.
Rawatan <i>Treatment</i>	:	• Admission: Admitted to the medical ward for close monitoring and intravenous antibiotics. • Antibiotics: Intravenous Ceftriaxone 1g once daily + Oral Azithromycin 500mg once daily. • Oxygen Therapy: Nasal prongs at 2-3 L/min to maintain SpO₂ > 94%. • IV Fluids: Normal Saline 1L over 8 hours, then maintenance fluids as needed. • Symptomatic Treatment: Paracetamol 1g orally/PRN for fever > 38°C. Cough suppressants as needed. • Monitoring: Regular vital signs monitoring (every 4 hours), oxygen saturation, and fluid balance. • Investigations: Repeat FBC and CRP in 48-72 hours. Review sputum culture results when available. • Patient Education: Advised on hydration, deep breathing exercises, and importance of completing antibiotic course.



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Rumusan prosedur yang dijalankan ke atas pesakit <i>Summary of procedures carried out on patient</i>	:	- Initial assessment in Emergency Department. - Chest X-ray performed on admission. - Blood investigations (FBC, CRP, Renal Profile, LFT) on admission and repeat FBC/CRP on Day 3. - Sputum collection for Gram stain and culture. - Intravenous cannula insertion for IV antibiotics and fluids. - Daily ward rounds and clinical assessments by medical team.
Preskripsi ubat-ubat yang diberikan kepada pesakit <i>Drugs and other medicaments prescribed to patients</i>	:	During Admission (2024-05-10 to 2024-05-14): - IV Ceftriaxone 1g OD - Oral Azithromycin 500mg OD - Paracetamol 1g PRN for fever - Salbutamol MDI 2 puffs PRN for cough (as needed) Upon Discharge (2024-05-14): - Oral Amoxicillin-Clavulanate 625mg TDS for 7 days - Oral Paracetamol 500mg PRN for fever/pain - Multivitamin OD for 1 month
Perkembangan keadaan pesakit sepanjang di bawah penjagaan doktor termasuk rawatan susulan <i>Progress of patient while under the care of the doctor including follow up</i>	:	Day 1 (2024-05-10): Admitted with fever, cough, SOB. Started on IV Ceftriaxone and Oral Azithromycin. SpO2 maintained at 94-95% on room air. Day 2 (2024-05-11): Fever subsided. Cough less productive. SpO2 96% on room air. Chest auscultation showed reduced crepitations. Day 3 (2024-05-12): Clinically stable. Afebrile. Cough significantly improved. Repeat FBC showed WBC $10.5 \times 10^9/L$, CRP 35 mg/L (down-trending). Day 4 (2024-05-13): Patient ambulating well. No respiratory distress. Eating and drinking well. Day 5 (2024-05-14): Clinically well, fit for discharge. Converted to oral antibiotics. Follow-up Plan: To review in Specialist Clinic (Internal Medicine) in 2 weeks (2024-05-28) with a repeat Chest X-ray.
Keadaan pesakit ketika berjumpa kali terakhir dengan doktor <i>Condition of the patient last seen by the doctor</i>	:	Patient is afebrile, clinically stable, and ambulating independently. Respiratory symptoms have significantly improved. No signs of respiratory distress. Chest clear on auscultation. Patient is well-hydrated and tolerating oral intake. Fit for discharge.

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Tarikh Date	:	2024-05-14
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Cuti sakit/sekolah

Medical certificate/school leave

Dari

From

2024-05-10

Hingga

To

2024-05-21 (12 days,
including weekend)

Surat kerja ringan yang diberikan

Light duty given

Dari

From

2024-05-22

Hingga

To

2024-06-04 (2 weeks
of light duty
recommended post-
MC)

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LAPORAN PERUBATAN MEDICAL REPORT

LAPORAN DISEDIAKAN OLEH
REPORT PREPARED BY :

Nama Name	:	Dr. Nurul Ain Binti Abdullah
No K/P I/C No.	:	850101-14-5678
Jawatan Designation	:	Pakar Perubatan Dalaman / Internal Medicine Specialist
Jabatan Department	:	Jabatan Perubatan Dalaman / Department of Internal Medicine
Kelulusan Qualification	:	MD (UM), M.Med (Internal Medicine) (UKM)
Tandatangan Signature	:	Dr. Nurul Ain
Tarikh Date	:	2024-05-15
Masa Time	:	10:30 AM

Cop Rasmi Hospital Official Hospital Stamp	:	Hospital Kuala Lumpur
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